

Bevacizumab and Zaltrap:

Alymsys, Avastin, Avzivi, Jobvene, Mvasi, Vegzelma, Zirabev, Zaltrap

Original Effective Date: 1/1/2023

Revision Date(s): 1/1/23, 7/1/23, 8/19/2024; 05/27/2025, 1/1/2026

Review Date: 1/1/2026

Policy type: Medical Necessity

Line of business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
Non-Preferred	Yes	Bevacizumab-awwb (Mvasi)	Q5107	Injection, bevacizumab-awwb, biosimilar, (Mvasi), 10 mg
Preferred	Yes	Bevacizumab-bvzr (Zirabev)	Q5118	Injection, bevacizumab-bvzr, biosimilar, (Zirabev), 10 mg
Non-Preferred	Yes	Bevacizumab (Avastin)	J9035	Injection, bevacizumab, 10 mg
Non-Preferred	Yes	Bevacizumab-tnjn (Avzivi)	J9999	Not otherwise classified, antineoplastic drugs
Non-Preferred	Yes	Bevacizumab-maly (Alymsys)	Q5126	Injection, bevacizumab-maly, biosimilar, (Alymsys), 10 mg
Non-Preferred	Yes	Bevacizumab-adcd (Vegzelma)	Q5129	Injection, bevacizumab-adcd (Vegzelma), biosimilar, 10 mg
Non-Preferred	Yes	Ziv-aflibercept (Zaltrap)	J9400	Injection, ziv-aflibercept, 1 mg
Non-Preferred	Yes	Bevacizumab – nwgd (Jobevne)	Q5160	Injection, bevacizumab-nwgd (Jobevne), biosimilar, 10 mg

- Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents (or their 505(b)(2) equivalents) prior to approval of a non-preferred product, when clinically appropriate.
- Preferred product requirements apply to all requests, both new starts and reauthorizations.
- Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
 - Clinical Pharmacology (Strong For); **OR**
 - Wolters Kluwer Lexi-Drugs® (Level A); **OR**
- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
 - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association, Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;
 - **Meeting abstracts and case reports are excluded from consideration;**
- Non-standard protocols may be approved based on unique clinical circumstances;
- Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network® (NCCN), or indication specific peer-reviewed literature;
- In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

Drug Name	HCPCS Code	Description
Avastin	J9035	Injection, bevacizumab, 10 mg
Zaltrap	J9400	Injection, ziv-aflibercept, 1 mg
Mvasi	Q5107	Injection, bevacizumab-awwb, biosimilar, (Mvasi), 10 mg
Zirabev	Q5118	Injection, bevacizumab-bvzr, biosimilar, (Zirabev), 10 mg
Alymsys	Q5126	Injection, bevacizumab-maly, biosimilar, (Alymsys), 10 mg

Vegzelma	Q5129	Injection, bevacizumab-adcd (Vegzelma), biosimilar, 10 mg
Avzivi	J9999	No otherwise classified, antineoplastic drugs
Jobeyne	Q5160	Injection, bevacizumab-nwgd (Jobeyne), biosimilar, 10 mg

References

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17. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed May 15, 2025. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
7/18/23	Added Universal Criteria, updated bevacizumab-adcd HCPCS, updated footer.
6/1/2024	Policy adopted by GEHA.
8/19/2024	Annual Review, addition of Avzivi.
5/27/2025	Addition of Jobvene, approved at P&T; NP/P reviewed.
6/5/2025	Document assessed for universal accessibility.
10/1/2025	Annual review by OncoHealth.
11/7/2025	Addition of J9400 Zaltrap to this policy to avoid overlap between the existing OncoHealth Zaltrap policy with the existing bevacizumab policy.
1/1/2026	Update Jobevne, HCPCS